Nab-paclitaxel · every 3 weeks

NCCP 00230 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

STEP 1

Before treatment

FBC, kidney and liver tests before treatment and each cycle. Review response after two cycles; the source specifies stopping if there is no response.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Nab-paclitaxel is infused over 30 minutes. Full visit time includes preparation and review.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Routine allergy premedicines are usually not required for this albumin-bound formulation. Low-risk antiemetic support is separate; previous reactions may change the plan.

Sources: NCCP 00230 v7 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.